



AH-QF-RD-04

DEPARTMENT OF RADIOLOGY

Patient's Details	: SAROJINI REDDY	F 63 Years
UHID	: AC14.0000038268	Ward/Bed No. : /
I.P.No./Bill No.	:	Received on : 05-Jul-2025
DRN	:	Reported On : 07-Jul-2025 11:45:48
Referring Doctor	: MANJULA RAO	

WHOLE BODY DIGITAL FDG PET CT SCAN**CLINICAL DETAILS:**

Diagnosed as carcinoma right breast elsewhere, for staging evaluation.

STUDY PROTOCOL:

Whole body PET scan coupled with IV contrast enhanced CT were obtained from vertex of skull to mid-thigh using a dedicated 128 slice Siemens BIOGRAPH Vision Digital PET CT scanner after 60 minutes of intravenous administration of ^{18}F -FDG. Multiplanar reconstructions were done. (P.S.: SUV max generated on digital TOF- PET CT scanners are known to be higher as compared to the SUV max generated on analog systems)

FINDINGS:**Brain**

No concerning focal enhancing / FDG avid lesion seen in the neuroparenchyma.

Note:- All brain lesions may not be evident on PET-CT imaging. MRI brain may be done if clinically warranted.*

Head and neck:

Orbits, naso-, oro- hypopharynx and larynx are normal.

Salivary & thyroid glands appear unremarkable.

No significant metabolically active lymph nodes are seen in the neck.

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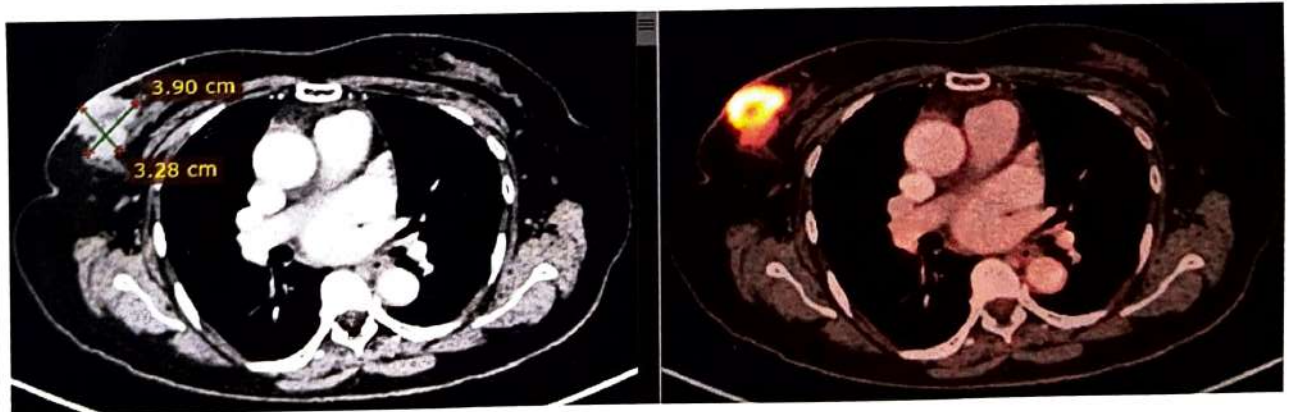
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Chest:

FDG avid enhancing irregular soft tissue dense mass is seen in the upper quadrants of right breast extending from 11 to 1 'o clock position, measuring ~ 3.9 x 3.2 cm (SUV max 17.5).

- The mass is seen infiltrating the overlying skin.
- No pectoralis muscle or chest wall infiltration.
- No other evident satellite lesion in right breast.

Left breast parenchyma appears unremarkable.



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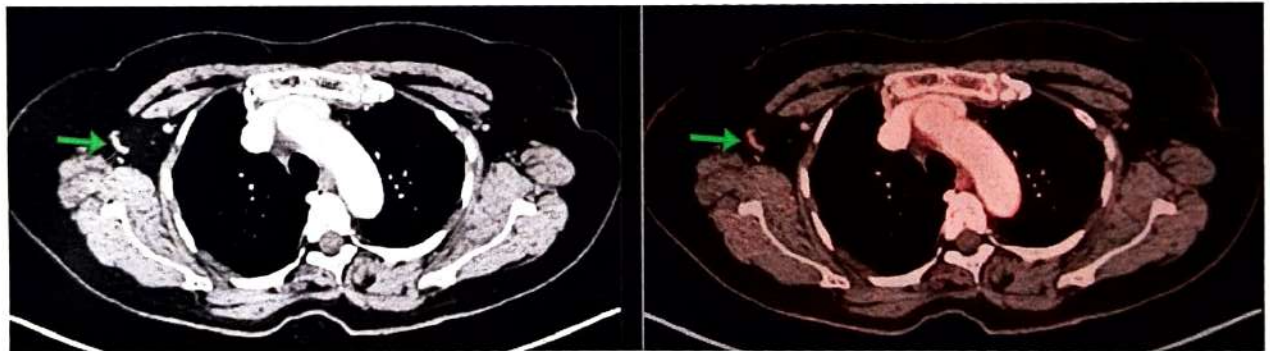
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Few prominent non FDG avid right level I axillary lymph nodes are seen with mildly thickened cortex and intact fatty hilum.

Few additional subcentimeter non FDG avid rounded level I & II right axillary lymph nodes are also seen with no discernible fatty hilum.

Lymph nodes with similar characteristics are also seen in left axilla – possibly reactive.



No significant FDG avid mediastinal, hilar or internal mammary lymph nodes seen.

No concerning soft tissue mass or nodules seen in the lungs.

A tiny subpleural calcified nodule seen in posterior basal segment of left lung lower lobe – granuloma.

No pleural or pericardial effusion is seen.

Note made of cardiomegaly with disproportionate enlargement of the both atria.

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Abdomen and Pelvis:

Liver shows physiological tracer activity. No intra hepatic biliary radicle dilatation. Portal vein is normal.

Status post cholecystectomy noted. Pancreas, spleen, adrenal glands, kidneys and urinary bladder demonstrate normal attenuation with physiologic FDG uptake. Small cortical cyst seen in right kidney.

Stomach, small bowel and large bowel loops appear grossly unremarkable showing physiological FDG uptake.

No peritoneal or omental deposits. No ascites.

No significant FDG avid abdominal, pelvic or inguinal lymphadenopathy noted.

Status post hysterectomy.

Musculo - Skeletal system:

No concerning focal lytic / sclerotic lesion or abnormal FDG uptake is seen in the bones or bone marrow.

Mild diffuse osteopenia noted.

Facetal arthrosis showing FDG uptake (L>R) seen in L4-L5 vertebrae – degenerative.

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IMPRESSION:

- FDG avid irregular soft tissue dense mass in the upper quadrants of right breast extending from 11 to 1 'o clock position – features in favor of primary malignant disease.**
 - The mass is seen infiltrating the overlying skin.
 - No pectoralis muscle or chest wall infiltration.
 - No other evident satellite lesion in right breast.
- Few prominent non FDG avid right level I axillary lymph nodes with mildly thickened cortex and intact fatty hilum. Few additional subcentimeter non FDG avid rounded level I & II right axillary lymph nodes with no discernible fatty hilum – these are indeterminate to characterize; suggested HPE correlation.**
- No significant metabolically active disease elsewhere in the wholebody survey.**

Please note that this report is a radiological professional opinion. It has to be correlated clinically and interpreted along with other investigations.

---END OF THE REPORT---


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